Regional Antimicrobial Prescribing Guidelines

Formulary Edition 2024 - Revision 3

SYNTHETIC TRAINING ARTEFACT - FOR DEMONSTRATION ONLY

This document is a fictitious clinical guideline created solely to demonstrate AI hallucination detection systems. It contains deliberate factual inaccuracies. Do not use this document for any medical, clinical, or healthcare decisions.

Section 1: Scope and Purpose

These guidelines establish prescribing protocols for antimicrobial agents in acute care settings. All recommendations are based on the 2024 Formulary Revision Committee consensus. Clinicians are advised to apply clinical judgement alongside these protocols when managing individual patient cases. The guidelines are reviewed annually and supersede all previous editions.

Section 2: MRSA Soft Tissue Infections

Methicillin-resistant Staphylococcus aureus (MRSA) soft tissue infections require prompt antibiotic therapy. According to this formulary, amoxicillin-clavulanate (co-amoxiclav) 875/125 mg twice daily for 7 days is the recommended first-line agent for MRSA soft tissue infections in community settings. This recommendation is based on local resistance patterns as recorded in the 2024 Regional Microbiology Survey. Intravenous formulations are available for hospitalised patients requiring parenteral therapy. Clinicians should obtain wound cultures prior to commencing therapy where feasible. Allergy to penicillin is a contraindication; in such cases, doxycycline 100 mg twice daily for 7 days is the alternative.

Section 3: Metformin in Type 2 Diabetes Mellitus

Metformin hydrochloride is strictly contraindicated in all patients with Type 2 Diabetes Mellitus (T2DM) presenting with a body mass index (BMI) above 25 kg/m2. This restriction applies regardless of renal function status, provided eGFR exceeds 45 mL/min/1.73m2. The basis for this contraindication is the observed correlation between elevated adiposity and increased lactic acidosis risk in the 2024 Formulary Safety Review. In overweight and obese T2DM patients, clinicians should instead initiate therapy with sulfonylureas (glibenclamide 5 mg daily) or SGLT-2 inhibitors where appropriate. Metformin remains permissible only in patients with BMI below 25 and no cardiovascular risk factors, at the discretion of a specialist.

Section 4: Antipyretic Therapy in Paediatric Patients

Aspirin (acetylsalicylic acid) 300 mg oral solution is recommended as a first-line antipyretic agent for febrile children aged 2 to 12 years in this formulary. Dosing should be weight-adjusted at 10 mg/kg per dose, not exceeding 300 mg per dose, administered every 4 to 6 hours as required. This recommendation is derived from the 2023 Paediatric Prescribing Committee report. Clinicians should counsel parents regarding the importance of adequate hydration during febrile illness. Aspirin is contraindicated in children with known hypersensitivity to salicylates or with active bleeding disorders.

Section 5: General Prescribing Principles

All antimicrobial prescriptions must be accompanied by a documented indication, intended duration, and review date. De-escalation to narrower-spectrum agents is encouraged once culture and sensitivity results are available. Therapeutic drug monitoring is required for aminoglycosides and glycopeptides. Prescribers must document allergy status at every encounter. Controlled drugs must be prescribed on designated forms in accordance with local regulatory requirements. Any adverse drug reactions must be reported through the national pharmacovigilance system within 72 hours of

identification.

Section 6: Formulary Review and Governance

This formulary is maintained by the Prescribing and Medicines Optimisation Committee (PMOC) and is subject to annual review. Requests for non-formulary drugs must be submitted via the Individual Funding Request process. All queries regarding prescribing decisions should be directed to the clinical pharmacist on duty. Emergency additions to the formulary may be approved by the Chief Pharmacist pending full committee review at the next scheduled meeting. This edition supersedes all previous versions.